A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

16 April 2026

Mr James Keck
Melbourne Colorectal Specialists

Dear Mr Keck,

Re: Mrs Patricia Wright
DOB: 1952-03-23
Address: 34 Pardoner Road, RYE, VIC, 3941

Summary

- **Gastroscopy identifying a 6 cm hiatus hernia.**
- **Colonoscopy revealing significant diverticulosis and a tubulovillous adenoma.**
- **Surveillance colonoscopy recommended in three years.**
- **Conservative management of reflux and visceral hypersensitivity.**
- **No action required by you.**

I had the pleasure of reviewing Patricia Wright in clinic following her recent gastroscopy and colonoscopy. I am pleased to report that the investigations did not reveal any evidence of malignancy or significant inflammatory conditions such as Coeliac disease.

The gastroscopy identified a moderate-sized hiatus hernia, measuring 6 centimetres. While Patricia does experience symptoms of reflux and heartburn, there is currently no evidence of erosive oesophagitis or Barrett's oesophagus. We discussed the long-term risks associated with a hernia of this size, including the potential for volvulus or obstruction. However, given the risks associated with surgical intervention in her age group—specifically bloating and potential nerve injury—we have agreed to proceed with conservative management.

Regarding the colonoscopy, the most significant finding was a tubulovillous adenoma, which was successfully resected. Given the histological subtype of this polyp, she requires a surveillance colonoscopy in three years. The procedure also confirmed significant diverticulosis, which likely accounts for her recurring abdominal symptoms. Biopsies of the anal lining were normal, and I

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Endoscopy locations
Epworth Freemasons Day Proc. Centre
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Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

attribute her previous reports of rectal bleeding to internal haemorrhoids identified during the examination.

Impression and Plan Patricia is currently managing her reflux symptoms with esomeprazole (Nexium), which remains an appropriate strategy. For her diverticular symptoms and associated visceral hypersensitivity, we discussed several options. I have previously prescribed mesalazine granules as an off-label trial to reduce colonic inflammation, though I have noted to Patricia that clinical evidence for its efficacy in recurrent diverticulitis is limited.

We also discussed the use of nerve-modulating pharmacotherapy to address her heightened intestinal sensitivity. While I explained how these medications can desensitise the bowel-brain signalling pathways, Patricia remains hesitant to start this therapy due to the potential side-effect profile. For now, she will continue with her current regimen, including Osmolax for bowel regulation. I have advised her that we can revisit the nerve-modulating options should her symptoms become more intrusive. We will review her on a clinical needs basis, and my office will send a reminder for her surveillance colonoscopy in three years.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Dr Peter Meggyesy

Investigation Results:

WRIGHT, PATRICIA ANNE : SVH 795519, SVP 604720

DOB: 23/03/1952 Age: 74 years

34 Pardoner Rd, Rye 3941

Results from 27/03/2026 ..... Episode# 26P213365 Test Hist (Histology - Routine)

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FINAL ANATOMICAL PATHOLOGY AAnat

Lab #: 2026B008232 **CLINICAL NOTES** Duo = N, Gas = N, gas polyp = ##Illegible##. Random colon = N, ascending colon. Anus bx=? Inflamed haemorrhoid vs? AIN. **MACROSCOPIC DESCRIPTION** 1. Specimen labelled "Gastric polyp". A intact tan polyp measuring 4 x 2 x 2 mm. Resection margin inked blue. Processed whole. A1. 2. Specimen labelled "Gastric bx". A mucosal fragment measuring 5 mm. A1. 3. Specimen labelled "Duodenal bx". Two mucosal fragments measuring both 2 mm. A1. 4. Specimen labelled "Descending colon polyp". Multiple polypoidal mucosal fragments measuring 1-3 mm. Please note: Associated vegetable matter not processed. A1. 5. Specimen labelled "Random colon bx". Four mucosal fragments measuring 2-4 mm. A1. 6. Specimen labelled "Anus bx". A mucosal fragment measuring 3 mm. A1. **MICROSCOPIC DESCRIPTION And DIAGNOSIS** **1. Gastric polyp - Features consistent with a fundic gland polyp. No evidence of dysplasia or malignancy. 2. Gastric - Gastric mucosal tissue with no significant pathological features. There is no evidence of intestinal metaplasia or gland atrophy. No Helicobacter pylori-like organisms are seen. No evidence of dysplasia or malignancy. 3. Duodenal - Duodenal mucosal biopsies with no significant pathological features. Morphological features of coeliac disease are not seen. No granulomas or parasites are identified. No evidence of dysplasia or malignancy. 4. Descending colon polyp - Tubulovillous adenoma with mild and moderate dysplasia (low grade). Severe dysplasia (high grade) or malignancy is not seen. 5. Random colon - Melanosis coli. No evidence of dysplasia or malignancy. 6. Anus - Hyperplastic polyp. No evidence of dysplasia or malignancy. Slides received for reporting 13/04/2026** Pathologist: Dr Cesar Salinas-LaRosa

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